

HOUSE No. 4335

The Commonwealth of Massachusetts

HOUSE OF REPRESENTATIVES, July 31, 2025.

The committee on Financial Services, to whom was referred the petition (accompanied by bill, House, No. 1151) of Kimberly N. Ferguson and others relative to healthcare insurance coverage for cognitive rehabilitation for individuals with an acquired brain injury, reports recommending that the accompanying bill (House, No. 4335) ought to pass.

For the committee,

JAMES M. MURPHY.

HOUSE No. 4335

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act relative to cognitive rehabilitation for individuals with an acquired brain injury.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 32A of the General Laws is hereby amended by inserting after
2 section 17Z the following section:-

3 Section 17AA. (a) For purposes of this section, the following terms shall have the
4 following meanings:-

5 “Acquired brain injury (ABI)” is any injury to the brain which occurs after birth and can
6 be caused by infectious diseases, metabolic disorders, endocrine disorders or diminished oxygen,
7 brain tumors, toxins, disease that affects the blood supply to the brain, stroke or a traumatic brain
8 injury.

9 “Cognitive communication therapy” treats problems with communication which have an
10 underlying cause in a cognitive deficit rather than a primary language or speech deficit.

11 “Cognitive rehabilitation therapy (CRT)” is a process of re-learning cognitive skills
12 essential for daily living through the coordinated specialized, integrated therapeutic treatments

which are provided in dynamic settings designed for efficient and effective re-learning following damage to brain cells or brain chemistry due to brain injury.

“Community reintegration services” provide incremental guided real-world therapeutic training to develop skills essential for an individual to participate in life: to re-enter employment; to go to school and engage in other productive activity; to safely live independently; and to participate in their community while avoiding re-hospitalization and long-term support needs.

“Functional rehabilitation therapy and remediation” is a structured approach to rehabilitation for brain disorders which emphasizes learning by doing, and focuses re-learning a specific task in a prescribed format, with maximum opportunity for repeated correct practice. Compensatory strategies are developed for those skills which are persistently impaired and individuals are trained on daily implementation. To ensure acquisition and use, focus is set on re-learning those skills essential for safe daily living in the environment in which they will be used: home and community settings.

“Medical necessity” or “medically necessary,” health care services that are consistent with generally accepted principles of professional medical practice.

“Neurobehavioral therapy” is a set of medical and therapeutic assessment and treatments focused on behavioral impairments associated with brain disease or injury and the amelioration of these impairments through the development of pro-social behavior.

“Neurocognitive therapy” is treatment of disorders in which the primary clinical deficit is in cognitive function which has not been present since birth and is a decline from a previously attained level of function.

“Neurofeedback therapy” is a direct training of brain function to enhance self-regulatory capacity or an individual’s ability to exert control over behavior, thoughts and feelings. It is a form of biofeedback whereby a patient can learn to control brain activity that is measured and recorded by an electroencephalogram.

“Neuropsychological testing” is a set of medical and therapeutic assessment and treatments focused on amelioration of cognitive, emotional, psychosocial and behavioral deficits caused by brain injury.

“Psychophysiological testing and treatment” is a set of medical and therapeutic assessment and treatments focused on psychophysiological disorders or physical disorders with psychological overlay.

“Post-acute residential treatment” includes integrated medical and therapeutic services, treatment, education, and skills training within a 24/7 real-world environment of care- a home and community setting. Maximum opportunity to for correct practice of skill in the context of use develops new neural pathways which ensure ongoing skill use and avoidance of re-hospitalization and long-term care.

(b) Any coverage offered by the commission to an active or retired employee of the commonwealth insured under the group insurance commission shall provide coverage for medically necessary treatment related to or as a result of an acquired brain injury. Medically necessary treatment shall include, but is not limited to, cognitive rehabilitation therapy; cognitive communication therapy; neurocognitive therapy and rehabilitation; neurobehavioral, neurophysiological, neuropsychological and psychophysiological testing and treatment; neurofeedback therapy; functional rehabilitation therapy and remediation; community

reintegration services; post-acute residential treatment services; inpatient services; outpatient and day treatment services; home and community based treatment. The benefits in this section shall not include any lifetime limitation or unreasonable annual limitation of the number of days or sessions of treatment services. A health benefit plan may not deny benefits for the coverage required based solely on the fact that the treatment or services are provided at a facility other than a hospital. Any limitations shall be separately stated by the commission. The benefits in this section shall not be subject to any greater deductible, coinsurance, copayments, or out-of-pocket limits than any other benefit provided by the commission.

(c) The commissioner of insurance shall require a health benefit plan issuer to provide adequate training to personnel responsible for preauthorization of coverage or utilization review for services under this section, in consultation with the Brain Injury Association of Massachusetts.

(d) Individual practitioners and treatment facilities shall be qualified to provide acute care and post-acute care rehabilitation services through possession of the appropriate licenses, accreditation, training and experience deemed customary and routine in the trade practice, including programs, regulated by the Executive Office of Health and Human Services, which provide services for people with brain injury and accredited programs by the Commission on Rehabilitation Facilities (CARF) Medical Rehabilitation with a Brain Injury Specialty Program.

SECTION 2. Chapter 175 of the General Laws is hereby amended by inserting before section 47CCC the following section:-

Section 47AAA. (a) For purposes of this section, the following terms shall have the following meanings:-

“Acquired brain injury (ABI)” is any injury to the brain which occurs after birth and can be caused by infectious diseases, metabolic disorders, endocrine disorders or diminished oxygen, brain tumors, toxins, disease that affects the blood supply to the brain, stroke or a traumatic brain injury.

“Cognitive communication therapy” treats problems with communication which have an underlying cause in a cognitive deficit rather than a primary language or speech deficit.

“Cognitive rehabilitation therapy (CRT)” is a process of relearning cognitive skills essential for daily living through the coordinated specialized, integrated therapeutic treatments which are provided in dynamic settings designed for efficient and effective re-learning following damage to brain cells or brain chemistry due to brain injury.

“Community reintegration services” provide incremental guided real-world therapeutic training to develop skills essential for an individual to participate in life: to re-enter employment; to go to school and engage in other productive activity; to safely live independently; and to participate in their community while avoiding re-hospitalization and long-term support needs.

“Functional rehabilitation therapy and remediation” is a structured approach to rehabilitation for brain disorders which emphasizes learning by doing, and focuses relearning a specific task in a prescribed format, with maximum opportunity for repeated correct practice. Compensatory strategies are developed for those skills which are persistently impaired and individuals are trained on daily implementation. To ensure acquisition and use, focus is set on re-learning those skills essential for safe daily living in the environment in which they will be used: home and community settings.

99 “Medical necessity” or “medically necessary,” health care services that are consistent
100 with generally accepted principles of professional medical practice.

101 “Neurobehavioral therapy” is a set of medical and therapeutic assessment and treatments
102 focused on behavioral impairments associated with brain disease or injury and the amelioration
103 of these impairments through the development of pro-social behavior.

104 “Neurocognitive therapy” is treatment of disorders in which the primary clinical deficit is
105 in cognitive function which has not been present since birth and is a decline from a previously
106 attained level of function.

107 “Neurofeedback therapy” is a direct training of brain function to enhance self-regulatory
108 capacity or an individual’s ability to exert control over behavior, thoughts and feelings. It is a
109 form of biofeedback whereby a patient can learn to control brain activity that is measured and
110 recorded by an electroencephalogram.

111 “Neuropsychological testing” is a set of medical and therapeutic assessment and
112 treatments focused on amelioration of cognitive, emotional, psychosocial and behavioral deficits
113 caused by brain injury.

114 “Psychophysiological testing and treatment” is a set of medical and therapeutic
115 assessment and treatments focused on psychophysiological disorders or physical disorders with
116 psychological overlay.

117 “Post-acute residential treatment” includes integrated medical and therapeutic services,
118 treatment, education, and skills training within a 24/7 real-world environment of care - a home
119 and community setting. Maximum opportunity for correct practice of skill in the context of use

develops new neural pathways which ensure ongoing skill use and avoidance of re-hospitalization and long-term care.

(b) The following shall provide coverage for medically necessary treatment related to or as a result of an acquired brain injury: (ii) any policy of accident and sickness insurance, as described in section 108, which provides hospital expense and surgical expense insurance and which is delivered, issued or subsequently renewed by agreement between the insurer and policyholder in the commonwealth; (ii) any blanket or general policy of insurance described in subdivision (A), (C) or (D) of section 110 which provides hospital expense and surgical expense insurance and which is delivered, issued or subsequently renewed by agreement between the insurer and the policyholder in or outside of the commonwealth; or (iii) any employees' health and welfare fund which provides hospital expense and surgical expense benefits and which is delivered, issued or renewed to any person or group of persons in the commonwealth. Medically necessary treatment shall include, but is not limited to, cognitive rehabilitation therapy; cognitive communication therapy; neurocognitive therapy and rehabilitation; neurobehavioral, neurophysiological, neuropsychological and psychophysiological testing and treatment; neurofeedback therapy; functional rehabilitation therapy and remediation; community reintegration services; post-acute residential treatment services; inpatient services; outpatient and day treatment services; home and community based treatment. The benefits in this section shall not include any lifetime limitation or unreasonable annual limitation of the number of days or sessions of treatment services. A health benefit plan may not deny benefits for the coverage required based solely on the fact that the treatment or services are provided at a facility other than a hospital. Any limitations shall be separately stated by the insurer. The benefits in this

section shall not be subject to any greater deductible, coinsurance, copayments, or out-of-pocket limits than any other benefit provided by the insurer.

(c) The commissioner of insurance shall require a health benefit plan issuer to provide adequate training to personnel responsible for preauthorization of coverage or utilization review for services under this section, in consultation with the Brain Injury Association of Massachusetts.

(d) Individual practitioners and treatment facilities shall be qualified to provide acute care and post-acute care rehabilitation services through possession of the appropriate licenses, accreditation, training and experience deemed customary and routine in the trade practice, including programs, regulated by the Executive Office of Health and Human Services, which provide services for people with brain injury and accredited programs by the Commission on Rehabilitation Facilities (CARF) Medical Rehabilitation with a Brain Injury Specialty Program.

SECTION 3. Chapter 176A of the General Laws is hereby amended by inserting after section 8DDD the following section:-

Section 8EEE. (a) For purposes of this section, the following terms shall have the following meanings:-

“Acquired brain injury (ABI)” is any injury to the brain which occurs after birth and can be caused by infectious diseases, metabolic disorders, endocrine disorders or diminished oxygen, brain tumors, toxins, disease that affects the blood supply to the brain, stroke or a traumatic brain injury.

“Cognitive communication therapy” treats problems with communication which have an underlying cause in a cognitive deficit rather than a primary language or speech deficit.

“Cognitive rehabilitation therapy (CRT)” is a process of re-learning cognitive skills essential for daily living through the coordinated specialized, integrated therapeutic treatments which are provided in dynamic settings designed for efficient and effective re-learning following damage to brain cells or brain chemistry due to brain injury.

“Community reintegration services” provide incremental guided real-world therapeutic training to develop skills essential for an individual to participate in life: to re-enter employment; to go to school and engage in other productive activity; to safely live independently; and to participate in their community while avoiding re-hospitalization and long-term support needs.

“Functional rehabilitation therapy and remediation” is a structured approach to rehabilitation for brain disorders which emphasizes learning by doing, and focuses re-learning a specific task in a prescribed format with maximum opportunity for repeated correct practice. Compensatory strategies are developed for those skills which are persistently impaired and individuals are trained on daily implementation. To ensure acquisition and use, focus is set on re-learning those skills essential for safe daily living in the environment in which they will be used: home and community settings.

“Medical necessity” or “medically necessary,” health care services that are consistent with generally accepted principles of professional medical practice.

“Neurobehavioral therapy” is a set of medical and therapeutic assessment and treatments focused on behavioral impairments associated with brain disease or injury and the amelioration of these impairments through the development of pro-social behavior.

“Neurocognitive therapy” is treatment of disorders in which the primary clinical deficit is in cognitive function which has not been present since birth and is a decline from a previously attained level of function.

“Neurofeedback therapy” is a direct training of brain function to enhance self-regulatory capacity or an individual’s ability to exert control over behavior, thoughts and feelings. It is a form of biofeedback whereby a patient can learn to control brain activity that is measured and recorded by an electroencephalogram.

“Neuropsychological testing” is a set of medical and therapeutic assessment and treatments focused on amelioration of cognitive, emotional, psychosocial and behavioral deficits caused by brain injury.

“Psychophysiological testing and treatment” is a set of medical and therapeutic assessment and treatments focused on psychophysiological disorders or physical disorders with psychological overlay.

“Post-acute residential treatment” includes integrated medical and therapeutic services, treatment, education, and skills training within a 24/7 real-world environment of care- a home and community setting. Maximum opportunity for correct practice of skill in the context of use develops new neural pathways which ensure ongoing skill use and avoidance of re-hospitalization and long-term care.

(b) Any contract between a subscriber and the corporation under an individual or group hospital service plan which is delivered, issued or renewed within the commonwealth shall provide coverage for medically necessary treatment related to or as a result of an acquired brain injury. Medically necessary treatment shall include, but is not limited to, cognitive rehabilitation

206 therapy; cognitive communication therapy; neurocognitive therapy and rehabilitation;
207 neurobehavioral, neurophysiological, neuropsychological and psychophysiological testing and
208 treatment; neurofeedback therapy; functional rehabilitation therapy and remediation; community
209 reintegration services; post-acute residential treatment services; inpatient services; outpatient and
210 day treatment services; home and community based treatment. The benefits in this section shall
211 not include any lifetime limitation or unreasonable annual limitation of the number of days or
212 sessions of treatment services. A health benefit plan may not deny benefits for the coverage
213 required based solely on the fact that the treatment or services are provided at a facility other
214 than a hospital. Any limitations shall be separately stated by the insurer. The benefits in this
215 section shall not be subject to any greater deductible, coinsurance, copayments, or out-of-pocket
216 limits than any other benefit provided by the insurer.

217 (c) The commissioner of insurance shall require a health benefit plan issuer to provide
218 adequate training to personnel responsible for preauthorization of coverage or utilization review
219 for services under this section, in consultation with the Brain Injury Association of
220 Massachusetts.

221 (d) Individual practitioners and treatment facilities shall be qualified to provide acute care
222 and post-acute care rehabilitation services through possession of the appropriate licenses,
223 accreditation, training and experience deemed customary and routine in the trade practice,
224 including programs, regulated by the Executive Office of Health and Human Services, which
225 provide services for people with brain injury and accredited programs by the Commission on
226 Rehabilitation Facilities (CARF) Medical Rehabilitation with a Brain Injury Specialty Program.

SECTION 4. Chapter 176B of the General Laws is hereby amended by inserting after section 4DDD the following section:-

Section 4EEE. (a) For purposes of this section, the following terms shall have the following meanings:-

“Acquired brain injury (ABI)” is any injury to the brain which occurs after birth and can be caused by infectious diseases, metabolic disorders, endocrine disorders or diminished oxygen, brain tumors, toxins, disease that affects the blood supply to the brain, stroke or a traumatic brain injury.

“Cognitive communication therapy” treats problems with communication which have an underlying cause in a cognitive deficit rather than a primary language or speech deficit.

“Cognitive rehabilitation therapy (CRT)” is a process of relearning cognitive skills essential for daily living through the coordinated specialized, integrated therapeutic treatments which are provided in dynamic settings designed for efficient and effective re-learning following damage to brain cells or brain chemistry due to brain injury.

“Community reintegration services” provide incremental guided real-world therapeutic training to develop skills essential for an individual to participate in life: to re-enter employment; to go to school and engage in other productive activity; to safely live independently; and to participate in their community while avoiding re-hospitalization and long-term support needs.

“Functional rehabilitation therapy and remediation” is a structured approach to rehabilitation for brain disorders which emphasizes learning by doing, and focuses re-learning a specific task in a prescribed format, with maximum opportunity for repeated correct practice.

248 Compensatory strategies are developed for those skills which are persistently impaired and
249 individuals are trained on daily implementation. To ensure acquisition and use, focus is set on re-
250 learning those skills essential for safe on daily living in the environment in which they will be
251 used: home and community settings.

252 “Medical necessity” or “medically necessary,” health care services that are consistent
253 with generally accepted principles of professional medical practice.

254 “Neurobehavioral therapy” is a set of medical and therapeutic assessment and treatments
255 focused on behavioral impairments associated with brain disease or injury and the amelioration
256 of these impairments through the development of pro-social behavior.

257 “Neurocognitive therapy” is treatment of disorders in which the primary clinical deficit is
258 in cognitive function which has not been present since birth and is a decline from a previously
259 attained level of function.

260 “Neurofeedback therapy” is a direct training of brain function to enhance self-regulatory
261 capacity or an individual’s ability to exert control over behavior, thoughts and feelings. It is a
262 form of biofeedback whereby a patient can learn to control brain activity that is measured and
263 recorded by an electroencephalogram.

264 “Neuropsychological testing” is a set of medical and therapeutic assessment and
265 treatments focused on amelioration of cognitive, emotional, psychosocial and behavioral deficits
266 caused by brain injury;

“Psychophysiological testing and treatment” is a set of medical and therapeutic assessment and treatments focused on psychophysiological disorders or physical disorders with psychological overlay.

“Post-acute residential treatment” includes integrated medical and therapeutic services, treatment, education, and skills training within a 24/7 real-world environment of care, – a home and community setting. Maximum opportunity for correct practice of skill in the context of use develops new neural pathways which ensure ongoing skill use and avoidance of re-hospitalization and long-term care.

(b) Any subscription certificate under an individual or group medical service agreement delivered, issued or renewed within the commonwealth shall provide coverage for medically necessary treatment related to or as a result of an acquired brain injury. Medically necessary treatment shall include, but is not limited to, cognitive rehabilitation therapy; cognitive communication therapy; neurocognitive therapy and rehabilitation; neurobehavioral, neurophysiological, neuropsychological and psychophysiological testing and treatment; neurofeedback therapy; functional rehabilitation therapy and remediation; community reintegration services; post-acute residential treatment services; inpatient services; outpatient and day treatment services; home and community based treatment. The benefits in this section shall not include any lifetime limitation or unreasonable annual limitation of the number of days or sessions of treatment services. A health benefit plan may not deny benefits for the coverage required based solely on the fact that the treatment or services are provided at a facility other than a hospital. Any limitations shall be separately stated by the insurer. The benefits in this section shall not be subject to any greater deductible, coinsurance, copayments, or out-of-pocket limits than any other benefit provided by the insurer.

(c) The commissioner of insurance shall require a health benefit plan issuer to provide adequate training to personnel responsible for preauthorization of coverage or utilization review for services under this section, in consultation with the Brain Injury Association of Massachusetts.

(d) Individual practitioners and treatment facilities shall be qualified to provide acute care and post-acute care rehabilitation services through possession of the appropriate licenses, accreditation, training and experience deemed customary and routine in the trade practice, including programs, regulated by the Executive Office of Health and Human Services, which provide services for people with brain injury and accredited programs by the Commission on Rehabilitation Facilities (CARF) Medical Rehabilitation with a Brain Injury Specialty Program.

SECTION 5. Chapter 176G of the General Laws is hereby amended by inserting after section 4VV the following section:-

Section 4WW. (a) For purposes of this section, the following terms shall have the following meanings:-

“Acquired brain injury (ABI)” is any injury to the brain which occurs after birth and can be caused by infectious diseases, metabolic disorders, endocrine disorders or diminished oxygen, brain tumors, toxins, disease that affects the blood supply to the brain, stroke or a traumatic brain injury.

“Cognitive communication therapy” treats problems with communication which have an underlying cause in a cognitive deficit rather than a primary language or speech deficit.

310 “Cognitive rehabilitation therapy (CRT)” is a process of relearning cognitive skills
311 essential for daily living through the coordinated specialized, integrated therapeutic treatments
312 which are provided in dynamic settings designed for efficient and effective re-learning following
313 damage to brain cells or brain chemistry due to brain injury.

314 “Community reintegration services” provide incremental guided real-world therapeutic
315 training to develop skills essential for an individual to participate in life: to re-enter employment;
316 to go to school or engage in other productive activity; to safely live independently; and to
317 participate in their community while avoiding re-hospitalization and long-term support needs.

318 “Functional rehabilitation therapy and remediation” is a structured approach to
319 rehabilitation for brain disorders which emphasizes learning by doing, and focuses re-learning a
320 specific task in a prescribed format, with maximum opportunity for repeated correct practice.
321 Compensatory strategies are developed for those skills which are persistently impaired and
322 individuals are trained on daily implementation. To ensure acquisition and use, focus is set on re-
323 learning those skills essential for safe daily living in the environment in which they will be used:
324 home and community settings.

325 “Medical necessity” or “medically necessary,” health care services that are consistent
326 with generally accepted principles of professional medical practice.

327 “Neurobehavioral therapy” is a set of medical and therapeutic assessment and treatments
328 focused on behavioral impairments associated with brain disease or injury and the amelioration
329 of these impairments through the development of pro-social behavior.

“Neurocognitive therapy” is treatment of disorders in which the primary clinical deficit is in cognitive function which has not been present since birth and is a decline from a previously attained level of function.

“Neurofeedback therapy” is a direct training of brain function to enhance self-regulatory capacity or an individual’s ability to exert control over behavior, thoughts and feelings. It is a form of biofeedback whereby a patient can learn to control brain activity that is measured and recorded by an electroencephalogram.

“Neuropsychological testing” is a set of medical and therapeutic assessment and treatments focused on amelioration of cognitive, emotional, psychosocial and behavioral deficits caused by brain injury.

“Psychophysiological testing and treatment” is a set of medical and therapeutic assessment and treatments focused on psychophysiological disorders or physical disorders with psychological overlay.

“Post-acute residential treatment” includes integrated medical and therapeutic services, treatment, education, and skills training within a 24/7 real-world environment of care— a home and community setting. Maximum opportunity for correct practice of skill in the context of use develops new neural pathways which ensure ongoing skill use and avoidance of re-hospitalization and long-term care.

(b) Any individual or group health maintenance contract shall provide coverage for medically necessary treatment related to or as a result of an acquired brain injury. Medically necessary treatment shall include, but is not limited to, cognitive rehabilitation therapy; cognitive communication therapy; neurocognitive therapy and rehabilitation; neurobehavioral,

neurophysiological, neuropsychological and psychophysiological testing and treatment; neurofeedback therapy; functional rehabilitation therapy and remediation; community reintegration services; post-acute residential treatment services; inpatient services; outpatient and day treatment services; home and community based treatment. The benefits in this section shall not include any lifetime limitation or unreasonable annual limitation of the number of days or sessions of treatment services. A health benefit plan may not deny benefits for the coverage required based solely on the fact that the treatment or services are provided at a facility other than a hospital. Any limitations shall be separately stated by the insurer. The benefits in this section shall not be subject to any greater deductible, coinsurance, copayments, or out-of-pocket limits than any other benefit provided by the insurer.

(c) The commissioner of insurance shall require a health benefit plan issuer to provide adequate training to personnel responsible for preauthorization of coverage or utilization review for services under this section, in consultation with the Brain Injury Association of Massachusetts.

(d) Individual practitioners and treatment facilities shall be qualified to provide acute care and post-acute care rehabilitation services through possession of the appropriate licenses, accreditation, training and experience deemed customary and routine in the trade practice, including programs, regulated by the Executive Office of Health and Human Services, which provide services for people with brain injury and accredited programs by the Commission on Rehabilitation Facilities (CARF) Medical Rehabilitation with a Brain Injury Specialty Program.